Narrated Accessibility Walk

CHA Cambridge Hospital (1493 Cambridge St), Cambridge, MA: approach from Spaulding, campus perimeter, main entrance and drop-off

Pilot report 5 | Recorded October 27, 2026

Part of an independent series of narrated walks that check whether people with disabilities can reach clinical research sites. Prepared by Uri Kartoun, PhD, Founder, DBbun LLC, Cambridge, MA | uri.kartoun@dbbun.com | dbbun.com

Contents

Summary....1
Walk details....2
What travelers would meet....2
Findings....3
A. Approach and campus perimeter....3
B. Main entrance, drop-off and garage....4
Evidence for the priority findings....6
Questions for the hospital and the city....6
Checking the AI-assisted analysis....7
Photo evidence....7
How this report was produced....9

Summary

A narrated walk continued along Cambridge St from Spaulding Rehabilitation Hospital to CHA Cambridge Hospital, around the outside of the campus, and then stayed at the main entrance for about half an hour to watch how people arrive, especially those dropped off by car or transport van.

- **25 items recorded:** 16 possible barriers, 3 rated high severity, 8 positive features and 1 note.
- **Arriving by car or van was the main difficulty seen.** The drop-off loop is too small for the traffic it gets: vans waiting, cars stopping in the travel lane, a short exit signal, and queues back to Cambridge St. People who most need a drop-off at the door, such as wheelchair users, are the ones it serves worst.
- **People with mobility aids take unsafe shortcuts.** A man with a rollator could not cross from the entrance toward the garage because there is no curb ramp on that line, and people with canes cut across the traffic island.
- **Positives:** automatic doors and benches at the entrance, clear signs, a visible Financial Assistance desk, an audible garage-exit warning, and patient parking capped at $10.
- **Scope:** outdoors only. By protocol, the walker does not enter care facilities without an appointment or an invitation.

Disclaimer. This report was produced with the help of AI tools (automatic speech transcription and AI-assisted image analysis; see “How this report was produced”) and was reviewed by the author. Errors and omissions are possible. Items are possible barriers observed on one day, not legal compliance determinations or findings of violation. Times and distances are estimates. Observations reflect conditions on October 27, 2026 and may have changed since. People and vehicles are not identified: faces, license plates and company logos are blurred. This is an independent project; it is not affiliated with or endorsed by the hospital.

Walk details

Date	October 27, 2026 (Tuesday), late morning, sunny and dry
Recordings	Video: Spaulding to CHA and around the campus (11 min); six short videos at the main entrance and drop-off (about 15 min in all, the longest 9 min 25 s). Audio only: three narrations at the entrance (15 min 23 s in all). 11 photos of the entrance drive, crossings and garage.
Route	Cambridge St from Spaulding to CHA, around the campus perimeter (Macht building, service area, Cambridge St frontage), then the main entrance, drop-off loop and garage entrance
Narrator	Uri Kartoun, PhD (DBbun LLC); assessed the route from the point of view of wheelchair, walker and cane users, blind and low-vision people, and people who tire easily. The walker does not use a mobility aid.
Why this site	CHA Cambridge Hospital is a public safety-net hospital and teaching site on the same street as Spaulding, serving many patients who rely on transport services. Its entrance shows how participants with disabilities would arrive for a visit.
Scope	Outdoors only; the lobby was described through the window.

What travelers would meet

What each group of travelers would likely meet, based on what the walker observed. These are not test results: no traveler with these disabilities made the trip, so each cell notes what was seen and, where it matters, what remains uncertain.

Traveler	Arriving on foot along Cambridge St	Arriving by car or van, and around the entrance	Main obstacles
Wheelchair (manual or power)	Seen: marked crosswalks; a patched but passable sidewalk (C10). Uncertain: the closed crossing near the entrance (C11)	Seen: a crowded drop-off loop; no curb ramp on the direct line to the garage (C15, C17). Uncertain: whether the step-free way around is signed	C15, C17
Walker, rollator or cane	Seen: a torn surface and many patches (C5, C10)	Seen: a man with a rollator could not cross (C15); people with canes cut across the island (C16)	C15, C16
Blind or low vision	Seen: a sign block with sharp edges beside the sidewalk (C9). Uncertain: tactile wayfinding	Seen: an audible garage-exit warning (positive); cars stopping in the lane and cutting around the island (C18)	C9, C18
People who tire easily or are sensitive to smoke	Seen: no benches between Spaulding and CHA (C1)	Seen: benches at the entrance; smoking by the tobacco-free signs (C12)	C1, C12

Findings

Severity notes appear in the barrier column; “positive” marks features that worked well. Conf. (confidence): H = seen and recorded on video or photo; M = described in the narration only, or seen once and not measured.

A. Approach and campus perimeter

#	Where	Possible barrier or feature	Observation	Most affected	Conf.	Suggested fix
1	Cambridge St, Spaulding to CHA (Lowell St and Leonard Ave area)	No benches on this stretch	Accessible crossing (positive), but no benches between the two hospitals.	People who need to rest	M	Add a bench
2	CHA campus, Emergency side and entrances	Clear signs (positive)	Clear “Emergency”, “Main Entrance”, “Not an entrance” and “Ambulance only” signs, with arrows; a signalized crossing nearby.	-	H	-
3	Path to a CHA building	Slightly steep path	Accessible but “a little bit steep”.	Wheelchair, walker	M	Check the slope; add a handrail if steep
4	CHA campus	Accessible parking; transport vans (positive)	Signed accessible parking spaces; several wheelchair-accessible transport vans in use on the campus.	-	H	-
5	Near the Macht building	Torn surface	A torn and damaged walking surface (“they will need to fix that”).	Wheelchair, walker, cane	M	Repair the surface
6	Macht building	Ramp, seating and signs (positive)	Wheelchair ramp; outdoor tables and seating; signs to Emergency and the Main Entrance.	-	H	-
7	Street beside the hospital, by a construction area	Delivery truck parked on the sidewalk	A delivery truck parked on the sidewalk next to a construction area, narrowing the path.	Wheelchair, walker, stroller, low vision	H	Loading zone off the sidewalk; enforcement
8	Side ramp near construction	Ramp looks unsafe	Because of the construction, the ramp looked unsafe; the walker did not use it.	Wheelchair	M	Close it with a signed alternative, or make it safe
9	Cambridge St, hospital frontage	Sign on a broken concrete block	A “CHA is tobacco free” sign mounted on a broken concrete block with very sharp edges, beside the sidewalk (“neglected… dangerous”).	Blind and low vision (a cane may miss it), children	H	Repair or remove the block
10	Cambridge St sidewalk along the hospital	Heavily patched sidewalk	Many patches (concrete or rubbery fill); passable for a wheelchair, but the walker would ask for the whole stretch to be replaced.	Wheelchair, walker	H	Replace the sidewalk (City of Cambridge or hospital, depending on ownership)

#	Where	Possible barrier or feature	Observation	Most affected	Conf.	Suggested fix
11	Crossing near the entrance	Crossing closed (temporary)	A crossing appeared closed with a caution sign; a pedestrian crossed beside it anyway.	Wheelchair, low vision	M	Signed alternative crossing while closed
12	Near entrances, by “CHA is tobacco free” signs	Smoking by the signs	Many tobacco-free signs (positive), but two people were seen smoking next to them, one about 30 ft from an entrance, while a person with a walker, helped by staff, passed slowly through the smoke. A smoking area exists near Cambridge St but was not used.	People with asthma or chemical sensitivity; slow walkers who cannot move away	M	Staff reminders; move the smoking area further from paths and entrances

B. Main entrance, drop-off and garage

#	Where	Possible barrier or feature	Observation	Most affected	Conf.	Suggested fix
13	Emergency entrance	Automatic doors and benches (positive)	Automatic doors and benches; a person with a cane walked out easily.	-	M	-
14	Lobby (seen through the window)	Registration and Financial Assistance (positive)	Registration and Financial Assistance desks visible from outside; about 20 people waiting, including people with rollators and crutches. Sun glare on the glass made signs hard to read from outside.	-	M	-
15	Crossing from the entrance toward the garage and the other hospital building	No curb ramp on the direct line (High)	A man who walks with great difficulty, using a rollator and helped by a companion, tried to cross here and could not: the curb on the far side is raised, with no ramp. The marked crosswalk lands further along, so people who cannot step up must go around.	Rollator, wheelchair, walker	H	Add a curb ramp on this line, or a sign at the entrance pointing to the step-free route
16	Traffic island in the entrance drive	Shortcuts across the island (High)	A woman with a cane, and later another person, cut across the island instead of using the marked crossings, stepping into the path of cars. The walker: the island is “kind of steep and dangerous”.	Cane, walker, low vision	M	Find where people cross and add a direct step-free crossing there; avoid barriers that would lengthen the walk

#	Where	Possible barrier or feature	Observation	Most affected	Conf.	Suggested fix
17	Entrance drop-off loop	Drop-off area too small (High)	During about half an hour at the entrance: up to 4 transport vans waiting at once; private cars and ride-share vehicles stopping in front of the entrance or in the travel lane; about 4 cars queuing in from Cambridge St; at most 3 cars can pass. Some drivers dropped off inside the garage instead. “Very difficult to drop patients properly.”	Wheelchair and walker users being dropped off; transport vans	H	Marked drop-off bays; a drop-off attendant at peak times; directions to drop-off in the garage
18	Travel lane by the island	Vehicles waiting in the travel lane by the island	Cars waited for several minutes in the travel lane beside the island, which is not a stopping area (the opposite curb is painted “Fire lane” and “No parking”), blocking cars and a taxi leaving the garage; drivers honked and one had to reverse.	Everyone leaving by car or van; pedestrians crossing between stopped cars	H	“No stopping” signs; enforcement
19	Exit signal onto Cambridge St	Short green for vehicles leaving	Timed from the video: about 10 seconds of green and a few seconds of amber, then about 75 seconds of red, with no turn on red (one full cycle; frames 3 seconds apart). Cars leaving the loop backed up into the drop-off area while they waited.	Vans and cars carrying patients	H	Review the signal timing with the city
20	Garage entrance	Steep ramp shared by cars and people	A steep ramp into the garage shared by cars and pedestrians (“definitely not for wheelchairs”); sign: “Caution pedestrian crosswalk top of ramp”.	Wheelchair, walker	M	Signed step-free pedestrian route to the garage elevators
21	Garage exit	Audible warning (positive)	“Caution vehicles exiting” sign with an audible warning.	-	H	-
22	Garage	Patient parking capped (positive)	Rates posted from free (first 10 minutes) to $40 for 6 to 24 hours; patients pay at most $10.	-	M	-
23	Side door	No push-button opener	A side door without a push-button opener; signs send people to the main entrance.	Wheelchair, walker	M	Opener, or a clear sign to the nearest accessible door
24	Entrance curb	Good drop-off (positive)	A ride-share driver stopped by the door with hazard lights, let a person with a walker out, then parked in the garage. A transport van loaded a wheelchair user by ramp at the curb when the space was free.	-	M	-

#	Where	Possible barrier or feature	Observation	Most affected	Conf.	Suggested fix
25	Entrance drive and crosswalk	Utility markings; temporary barriers	Fresh yellow utility markings on the crosswalk and drive, which may mean work is planned, and orange plastic barriers in the drive (purpose not clear).	-	H	If work is planned, add the missing curb ramp (item 15) at the same time

Evidence for the priority findings

Where each priority finding can be checked in the recordings. The recordings are kept by the author and are not published, because they show faces and license plates.

#	Finding	Where to find it (file, time)
9	Sign on a broken concrete block	Video IMG_6455, 9:47–10:17
12	Smoking next to the tobacco-free signs	Audio camelia_ave.m4a, 0:00–1:50; audio hovey_ave.m4a, 0:00–0:13 (narration only)
15	No curb ramp; a man with a rollator could not cross	Seen and narrated as it happened: audio camelia_ave.m4a, 2:35–3:34. Explained on site afterwards: video IMG_6457, 0:08–1:13. The curb: photo IMG_6465. The man himself was not filmed.
16	Shortcuts across the traffic island	First person: audio hovey_ave.m4a, 3:41–4:53, and video IMG_6456, 0:15 –1:14. Second person: video IMG_6472, 5:24–5:47. The island: photo IMG_6469.
17	Drop-off loop too small	Audio cha_cambridge_hospital.m4a, 0:00–3:38; videos IMG_6458 to IMG_6460; video IMG_6472, 0:00–4:59; photo IMG_6467
18	Vehicles waiting in the travel lane by the island	Videos IMG_6458 (0:00–0:39), IMG_6459, IMG_6460; video IMG_6472, 0:00–3:31
19	Short exit green onto Cambridge St	Video IMG_6472: signal visible from 7:24 to 8:57 (frames 3 seconds apart); narration 6:23–9:17

Questions for the hospital and the city

- Could the hospital mark drop-off bays, add “no stopping” signs along the island, and post an attendant at busy times?
- Could a curb ramp be added on the line between the entrance and the garage and the other building (item 15)? The fresh utility markings suggest work may be planned there.
- Could a direct, step-free crossing be added where people already cut across the traffic island (item 16), rather than barriers that would lengthen their walk?
- Could the City of Cambridge review the exit signal timing onto Cambridge St (item 19) and the condition of the Cambridge St sidewalk along the hospital (item 10)?
- How do study teams tell participants with disabilities where to be dropped off (at the door or in the garage), and is there an accessible route from the garage?
- Could the smoking area be moved further from entrances and paths, and the broken sign block (item 9) be repaired?

Checking the AI-assisted analysis

The item list was drafted with AI help from the transcribed narration, the face-blurred frames and the photos, and reviewed by the walker. Corrections from that review: the audio files' names were the phone's automatic location labels (two street names), but all three narrations were at CHA; the transcription heard "CJ" for CHA and "stroller" for a rollator; and an unfamiliar word in the narration was confirmed by the walker to mean crutches. The video first thought to show Spaulding mostly covers CHA, so it is used here. The key claims in the narration were checked against the videos and photos: the raised curb without a ramp (Figure 1), the traffic island (Figure 3), the crowded drop-off loop (Figure 4) and a car waiting in the travel lane by the island (Figure 5). License plates and van-company logos were blurred by hand, and the narration's mention of a plate number was left out.

Photo evidence

Photos and video frames; faces, license plates and company logos are blurred.

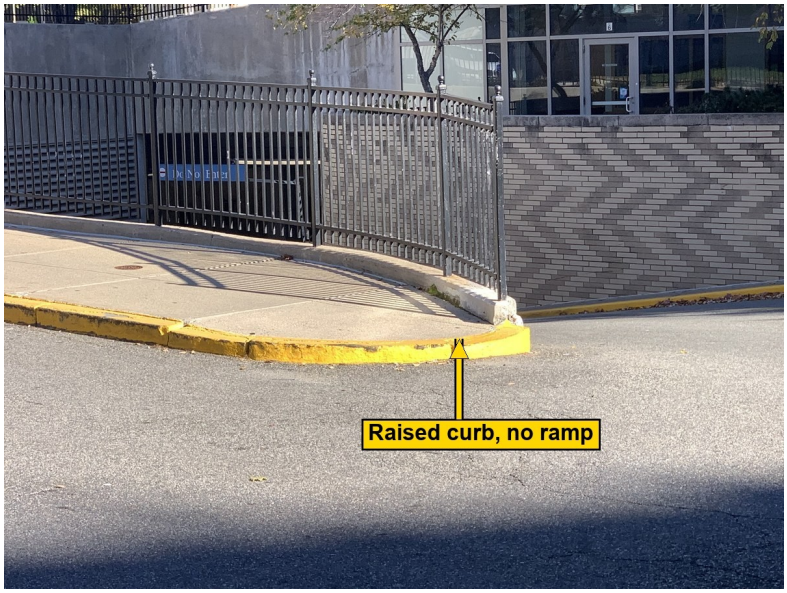


Figure 1. The raised curb with no ramp on the direct line from the entrance toward the garage and the other building, where a man with a rollator could not cross (item 15).



Figure 2. The same area from the entrance side: the marked crosswalk (right) and the curb toward the other building (left); fresh yellow utility markings on the road (items 15 and 25).



Figure 3. The traffic island in the entrance drive, with a “Keep right” sign, where people with canes cut across (item 16).



Figure 4. The drop-off loop: transport vans and cars stopped along the island in front of the entrance (item 17).



Figure 5. A car (the dark SUV) standing for several minutes in the travel lane beside the island, which is not a stopping area; the cars behind it and traffic leaving the garage could not get past (item 18).

How this report was produced

- **Capture:** seven narrated smartphone videos, three audio recordings and 11 photos made by the author.
- **Local processing on the author's computer:** audio extracted with ffmpeg (imageio-ffmpeg 7.1); narration transcribed with faster-whisper, model medium.en, on a local GPU; one frame sampled every 3 seconds; faces blurred with OpenCV 4.14 before any image left the computer; license plates and logos blurred by hand in the figures.
- **AI review:** the blurred frames, the photos and the transcripts were reviewed by Claude (Anthropic), model claude-opus-5-5, which drafted the item list and this report. The author reviewed the findings.
- **Limitations:** one walker on one dry morning; signal timings, distances and vehicle counts are estimates; the interior was not assessed; items are possible barriers, not legal compliance determinations.